Letrozole · daily hormone therapy

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.
- No routine antiemetic, steroid premedication or G-CSF is required by this endocrine monotherapy. Ask for individual symptom support.
- Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.
- The usual prescription is once daily, but severe liver impairment can require a different frequency. Use the frequency on your prescription and confirm any change with the prescriber or pharmacist. An already reduced or unusual schedule must be reviewed before it is changed. Confirmed menopause is needed for this monotherapy; any ovarian suppression has its own prescription and monitoring.
- NCCP no renal adjustment including haemodialysis expected; HPRA creatinine clearance (CrCl) <10 lacks sufficient data and requires careful benefit/risk review. Child–Pugh A/B no adjustment. C: NCCP 50% original average dose = 2.5 mg every other day; HPRA close supervision without this numerical instruction. Reconcile current reduced/custom schedule first.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00371 v4 · Monitoring and treatment map